



| ALLIED HEALTH – SOCIAL WORK<br>CLINICAL PRACTICE GUIDELINE                        |                                 |
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| Child Protection – Social Work                                                    |                                 |
| Scope (Staff):                                                                    | Head of Department, Social Work |
| Scope (Area):                                                                     | Social Work, Allied Health      |
| This document should be read in conjunction with the <a href="#">Disclaimer</a> . |                                 |

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## Aim

To provide clinical guidance for social workers when assessing child protection risks to unborn and newborn children.

## Key points

The wellbeing, care and protection and rights of children are paramount when making decisions about child protection, abuse and neglect. This is consistent with the [United Nations Convention on the Rights of the Child](#), internationally accepted practice and legislated in WA via the [Children and Community Services Act 2004](#).

Psycho/social assessment and intervention is to be informed by the [WA Health; Guidelines for Protecting Children 2020](#).

The role of coordinating the assessment, planning and intervention in families where child protection concerns have been identified is the responsibility of the Social Work Department.

The Social Work Department's response is to be consistent with [WNHS Policy Child Protection and Mandatory Reporting of Child Sexual Abuse](#) and reciprocal child protection procedures outlined in the Bilateral Schedule between The Department of Communities, Child Protection and Family Support and the Child and Adolescent Health Service, North Metropolitan Health Service, South Metropolitan health Service, East Metropolitan Health Services and WA Country Health Service (currently under review).

WNHS recognises the duty of care to the unborn child or newborn and the mother, father and family.

The infant is always considered in the context of the family, their culture and the community and all efforts will be made to maintain the integrity of the family. Best practice aims to manage risks and maximise the family's abilities to care for their children.

In the context of this service the responsibility is to promote the physical, psychological and emotional health and well-being of the patient i.e. the pregnant woman is a priority antenatally.

A referral to Department of Communities, Child Protection and Family Support (Communities), with the possibility of statutory intervention for an unborn/newborn, requires careful assessment and consultation with the Head of Department. Comprehensive documentation is required to ensure the rights and needs of the unborn/ newborn baby, and the interests of the family to fair and respectful processes.

### **The Role of the Social Worker**

- Engage the woman in positive relationships with health care providers in the Hospital.
- Facilitate access of the pregnant woman to antenatal care.
- Conduct a psychosocial assessment of the woman, her family and social circumstances.
- In consultation with other colleagues and Social Work Head of Department to form a professional opinion about the risk of abuse or neglect to the baby.
- Ensure early and detailed inter-agency planning to minimise crises and empower the family and their network to ensure the safety of the baby.

## Assessment

The aim of the assessment is to identify psycho-social issues that will impact on the parent's capacity to access appropriate antenatal care and services to parent a newborn safely. The social worker will develop intervention plans in partnership with the patient and other relevant service providers. The process may consist of a combination of the following:

- review medical and social work notes for information that relates to the child protection risk criteria.
- consult with relevant medical, nursing and allied health staff (eg midwife, neonatal nurse, shift coordinator, clinical nurse specialist, clinical psychologist, psychiatrist, resident, registrar or consultant obstetrician and paediatrician) within WNHS and with external health providers where identified
- contact Communities, (email: [CPDUTY@communities.wa.gov.au](mailto:CPDUTY@communities.wa.gov.au); or phone: 1800 273 889) to obtain any background of previous involvement with the family if assessed as necessary.
- interview with mother/parents by (social worker and/or other hospital staff)
- identify existing stressors - emotional, social and physical, and assess coping mechanisms of caregivers
- identify existing strengths and supports (social and physical), or lack of supports
- review any of the 'at risk' criteria that need further as per assessment guidelines below
- assess attitudes and expectations of pregnancy, childcare and parenthood
- if appropriate, and ideally with the patient's approval and consent, seek information, from significant others such as extended family and health/welfare/justice/ external agency staff etc.
- assess the parent's understanding of concerns for and risks to the baby and the parent's motivation to decrease the risks where identified
- assess risk to staff inside and outside the hospital and alert Security and staff to concerns you may have by:
  - documenting in the record
  - formally advising security
  - utilising the WNHS Complex Care guideline
- discuss assessment and plan with the relevant clinical team and the Head of Department Social Work and record planning in the patient's medical record
- ensure patient is "flagged for complex care if appropriate as per the [WNHS Complex Care Guidelines](#) (currently under review)

The outcome of the assessment process will be to inform a professional opinion about the needs of the woman and her unborn child's risk of abuse or neglect. A

thorough assessment of the protective and resiliency factors need to be made in conjunction with the risk factors. This is to be undertaken in conjunction with other staff and with input from external agencies where relevant.

Assessment includes these indicators of risk as presented by either parent:

#### **Antenatal:**

- other family members (child or adult) are or have been abused or neglected,
- patient has other children not in her care/in care of Communities
- history of mental health problems
- history of substantial substance abuse
- significant intellectual disability
- significant environmental deprivation or social instability/transience
- current or recent history of domestic violence
- patient or partner aggressive or hostile-provoked or unprovoked
- history of criminal or aggressive behaviour
- either parent or adult in household – convicted of an offence against a child
- poor attendance at antenatal clinic
- refusal to engage with services

#### **Post Birth:**

- an unwillingness to prepare for or learn about parenting
- unrealistic expectations of the baby or child
- rejection or significant lack of attachment to the baby
- child with special needs
- ongoing mother-crafting concerns reported by midwifery staff, including long periods of time away from ward.
- gather additional information as per above assessment criteria

#### **Intervention**

Plan your intervention according to the assessment, Intervention may include the following:

- Share your concerns with the mother/caregivers and working with them to plan strategies or outline your planned strategies to enhance their parenting skills and reduce stressors.
- Offer support services e.g local community services and family support networks, 24-hour crisis number, Ngala, advising Child Health Nurse.
- Establish kin and service networks that can provide some input and continue some assessment of needs eg extended family, especially grandparents,

parents' siblings, local Child Health Nurse, local community health social worker.

## **Referral to Other Agencies**

Seek the parent's permission for referrals to outside agencies and if possible, they should sign consenting to this. Referrals should include the reason for referral and request for other agency to be involved, problems/psychosocial assessment, action to date, plan.

## **Reasons to Notify the Department of Communities, Child Protection and Family Support (CPFS)**

- cases where an assessment has been completed and significant risk to unborn is identified, a referral to Communities is required.
- Cases where the patient has older children in care of Communities.

Early Referral and planning is crucial to achieve the best possible outcome for families.

## **Recording in the Patient Medical Record**

As per SW Record keeping and documentation guideline;. Use the ISOBAR format which should be completed as soon as practicable. New information should be documented in the medical record as progress notes attached to the correct episode of care. This will include updated assessments and plans. Once the baby is born relevant information is to be documented in the baby's medical record as [NMHS Policy – Clinical Documentation](#)

Because of the legal implications of child abuse, notes may be subpoenaed to Court to be used as evidence. They must therefore be an accurate reflection of what has occurred i.e. facts separated from assessment (rather than opinions).

## **Pre-Birth Planning**

Once a referral is made to Communities and accepted, refer to the Bilateral Schedule (currently under review);, for process of collaborative assessment and intervention.

All patients open to Communities in the pre-birth period should be notified to the Pre-birth planning team ([PrebirthPlanning@communities.wa.gov.au](mailto:PrebirthPlanning@communities.wa.gov.au))

Pre-birth meetings in the first instance are to be scheduled face to face in the Family Interview Room at KEMH, or as per patient's preference. Microsoft Teams can be used where attendees are in country locations.

Social Work to liaise with security if there are security concerns.

## Multi-Disciplinary Complex Care Planning Meetings

Patients are to be flagged by Social Work with the Complex Care Multidisciplinary Team as per the [WNHS Complex Care guidelines](#) (currently under review) when:

- When newborn baby is not to be placed with patient (mother of baby)
- There are concerns about placing baby on ward with patient (mother of the baby).
- when there is a risk or threat has been made likely to lead to volatile/angry/violent reaction toward WNHS and Communities staff
- when the patient has not attended or engaged antenatal care or collaborative planning meetings
- when the circumstances are such or the patient's behaviour suggests that the baby may need to go to the special care nursery

## Discharge of baby against medical advice

The risk needs to be assessed if a mother intends to discharge herself and the baby against medical advice. The Paediatric Consultant has the final authority of this and can enact Section 40 under the [Children and Community Services Act 2004](#) to prevent the newborn baby from being discharged against medical advice. Social Work assessment can assist in the judgment of risk. See [Social Work Clinical Practice Guideline: Discharge of a Neonate Against Medical Advice – Social Work](#).

## At Risk Data Collection

The Social Work Department collects its own statistics on all babies/children who were assessed as being at risk of child abuse or neglect. This is a clinical audit tool about both process and outcome. The data base is de-identified to ensure patient confidentiality. The purpose is to:

- establish the incidence of women who present to the Hospital with complex factors that may pose a risk to the baby;
- track the interventions undertaken to address the concern of risk to the baby;
- assist with service provision and planning;
- describe the trends in demographics and needs of this population

## Disclosure of Information to Other Agencies

In most instances the relevant exchange of information would be conducted in the course of the interagency collaborative meetings. Information may be shared with another agency with the permission of the patient if the KEMH Social Work department patient consent for release of information to an external agency form (located in the Social Work W Drive) has been signed by the patient. Ensure form is uploaded to DMR in the correspondence section.

The legislation enabling exchange of information between prescribed authorities is covered under Sections 23, 28, 33A and 33B of the [Children and Community Services Act 2004](#).

Privacy laws do not allow disclosure of the mother's medical condition without her consent. This includes requests for information about the mother's medical condition (such as Hepatitis C status or any toxicology results).

The foster families can be told that in the case of blood borne viruses, universal precautions should be used until the status of the baby is known which is likely by 18 months of age, but the mother's medical history must not be disclosed.

## References

[Children and Community Services Act 2004 \(WA\)](#)

[Bilateral Schedule: Interagency Collaborative Process When an Unborn or Newborn Baby is Identified at Risk of Abuse and/or Neglect](#) (currently under review)

## Related policies

[WA Health Guidelines for Protecting Children 2020](#)

[WA Health Mandatory Policy MP 0166/21 Mandatory Reporting of Child Sexual Abuse Training Policy](#)

[NMHS Policy: Patient Identification and Procedure Matching](#)

[NMHS Policy: Clinical Documentation](#)

[NMHS Policy: Healthcare Record Management Policy](#)

[NMHS Policy: Healthcare Record Access, Use and Disclosure Policy](#)

## Related WNHS policies, procedures and guidelines

[WNHS Policy: Child Protection and Mandatory Reporting of Child Sexual Abuse](#)

[WNHS Procedure: Patient Identification](#)

[Allied Health Clinical Practice Guideline: Working with Aboriginal Families – Allied Health](#)

[Social Work Clinical Practice Guideline: Anonymity/Privacy](#)

[Social Work Clinical Practice Guideline: Discharge of Neonate Against Medical Advice](#)

[Social Work Clinical Practice Guideline: Working with Aboriginal Families – Social Work](#)

[Social Work Clinical Practice Guideline: Working with Obstetric Patients – Social Work](#)

Social Work Clinical Practice Guideline: Documentation and Record Keeping (link to follow)

Social Work Clinical Practice Guideline: Family and Domestic Violence (link to follow)

Social Work Clinical Practice Guideline: Working with Security (link to follow)

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The health impact upon Aboriginal people has been considered; Impact and Statement Declaration (ISD) 2873.

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